

Marufah Odetundun, MD — Portfolio Sample

SOAP Note Documentation Improvement Example

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DOCUMENTATION IMPROVEMENT DEMONSTRATION

BEFORE: Incomplete Documentation
Patient: 65yo M CC: Diabetes follow-up HPI: Blood sugars high. Not taking meds regularly. PE: Unremarkable A/P: Continue metformin. RTC 3 months.
ISSUES WITH THIS DOCUMENTATION:
■ Missing specific blood glucose values and patterns
■ No adherence barrier assessment or patient education
■ No review of systems for diabetic complications
■ Incomplete physical exam (no vital signs, no diabetic foot exam)
■ No ICD-10 codes with specificity
■ Missed new diabetic complication (neuropathy)
■ No quality metric compliance (overdue eye exam not addressed)
■ Inadequate E/M level documentation (would support 99213 at best)

AFTER: COMPREHENSIVE ENHANCED DOCUMENTATION

PATIENT: 65-year-old male

CHIEF COMPLAINT: Type 2 diabetes mellitus, uncontrolled

HISTORY OF PRESENT ILLNESS:

Patient presents for diabetes follow-up. Reports blood glucose readings 200-300 mg/dL over the past month despite prescribed metformin 1000 mg BID. Admits to medication non-adherence due to cost concerns and forgetfulness. Denies polyuria, polydipsia, or weight changes. No hypoglycemic episodes reported.

REVIEW OF SYSTEMS:

Reports bilateral foot tingling (new symptom, onset 2 weeks ago). Denies vision changes, chest pain, or shortness of breath. No recent infections or wounds.

PAST MEDICAL HISTORY:

- Type 2 diabetes mellitus (diagnosed 2015)
- Hypertension
- Hyperlipidemia

CURRENT MEDICATIONS:

- Metformin 1000 mg BID (non-adherent - taking irregularly)
- Lisinopril 10 mg daily
- Atorvastatin 20 mg daily

ALLERGIES: No known drug allergies

SOCIAL HISTORY:

Non-smoker. Occasional alcohol use. Sedentary lifestyle. Retired accountant.

PHYSICAL EXAMINATION:

Vital Signs:

- Blood Pressure: 142/88 mmHg
- Heart Rate: 78 bpm
- BMI: 32 kg/m² (obese)
- Weight: 210 lbs (stable from last visit)

General: Alert and oriented, no acute distress

HEENT: Normocephalic, atraumatic. Last diabetic eye exam 8 months ago (now overdue for annual exam per ADA guidelines).

Cardiovascular: Regular rate and rhythm. No murmurs, rubs, or gallops.

Extremities:

- Pulses: Dorsalis pedis and posterior tibial pulses palpable bilaterally
- Skin: No ulcerations or lesions noted
- **Neurological: Diminished sensation to 10g monofilament testing bilaterally in feet (abnormal)**
- No deformities, full range of motion

ASSESSMENT & PLAN:

1. Type 2 Diabetes Mellitus, with hyperglycemia (ICD-10: E11.65)

- Current A1c unknown - ordered today for trending
- Blood glucose poorly controlled (200-300 mg/dL) due to non-adherence
- Discussed medication adherence barriers:
 - Cost concerns: Provided information on patient assistance programs
 - Forgetfulness: Patient will trial pill organizer and set phone reminders
- Consider adding second-line agent (e.g., GLP-1 agonist or SGLT2 inhibitor) if A1c >8%
- Diabetes education referral placed - patient amenable
- Dietary counseling provided - encouraged low glycemic index diet
- Encouraged daily exercise (30 min walking)

2. Diabetic Peripheral Neuropathy (ICD-10: E11.42) - NEW DIAGNOSIS

- New diagnosis based on:
 - Patient-reported bilateral foot tingling (2-week duration)
 - Abnormal monofilament testing bilaterally
- Likely related to poor glycemic control
- Plan:
 - Will monitor symptoms at follow-up
 - Consider gabapentin 300mg QHS if symptoms worsen or affect sleep
 - Reinforced critical importance of glucose control to prevent progression
 - Educated on diabetic foot care and daily self-inspection

3. Hypertension, uncontrolled (ICD-10: I10)

- BP 142/88 mmHg today (goal <130/80 for diabetic patient per ACC/AHA guidelines)
- Continue current regimen: Lisinopril 10mg daily
- Plan: Recheck BP in 2 weeks
- If still elevated, will titrate lisinopril to 20mg daily
- Home BP monitoring encouraged

4. Hyperlipidemia (ICD-10: E78.5)

- Continue atorvastatin 20mg daily
- Lipid panel ordered for monitoring

5. Obesity (ICD-10: E66.9)

- BMI 32 kg/m²
- Weight management counseling provided
- Referred to nutritionist for weight loss support

6. Overdue Diabetic Eye Examination - QUALITY METRIC

- Last exam 8 months ago (annual exam overdue per ADA guidelines)
- Ophthalmology referral placed today
- Patient educated on diabetic retinopathy risk
- Importance of annual dilated eye exams reinforced

ORDERS PLACED:

- HbA1c
- Comprehensive Metabolic Panel
- Lipid Panel
- Urine microalbumin/creatinine ratio
- Ophthalmology referral
- Diabetes education referral
- Nutrition referral

PATIENT EDUCATION PROVIDED:

- Medication adherence importance
- Diabetic foot care
- Blood glucose monitoring technique
- Warning signs of hypoglycemia
- Diet and exercise recommendations

FOLLOW-UP PLAN:

- Return to clinic in **2 weeks** for blood pressure recheck
- Return to clinic in **4 weeks** for A1c results review and medication adjustment if needed
- Patient verbalized understanding of plan and agrees
- All questions answered

TIME SPENT: 25 minutes face-to-face with patient

- >50% spent on counseling and coordination of care

E/M LEVEL: 99214 (Established patient, moderate complexity)

Supported by: comprehensive HPI, detailed ROS, detailed exam, moderate MDM (multiple diagnoses, prescription management, new problem with workup)

VALUE ADDED THROUGH DOCUMENTATION IMPROVEMENT

IMPROVEMENT CATEGORY	SPECIFIC VALUE ADDED
Clinical Care Quality	✓ Identified new diabetic complication (peripheral neuropathy - E11.42) ✓ Addressed adherence barriers with patient-centered solutions ✓ Comprehensive management of multiple chronic conditions
Coding & Revenue	✓ Supports 99214 vs original 99213 (approximately \$40-60 additional revenue) ✓ Proper ICD-10 coding with specificity for complications ✓ Time-based coding documentation included for potential upgrade
Quality Metrics	✓ Diabetic eye exam referral placed (HEDIS quality measure) ✓ Blood pressure control addressed (HEDIS HTN measure) ✓ HbA1c testing ordered (diabetes quality measure) ✓ Microalbumin screening ordered (diabetic nephropathy screening)
Risk Adjustment	✓ Documented diabetic peripheral neuropathy (HCC 18 - adds RAF value) ✓ Uncontrolled diabetes with complications properly coded ✓ Multiple chronic conditions documented with complexity
Compliance & Legal	✓ Comprehensive documentation supports medical necessity ✓ Patient education thoroughly documented ✓ Shared decision-making evident ✓ Follow-up plan clearly stated
Patient Safety	✓ Diabetic foot care education provided ✓ Hypoglycemia warning signs reviewed ✓ Clear follow-up intervals established ✓ Safety netting for worsening symptoms